THE STATE OF NEW HAMPSHIRE
SUPREME COURT

No. 2024-0396

State of New Hampshire
v.
Robert F. Allore

Interlocutory Appeal Pursuant to Rule 8 from the Rockingham County Superior Court

BRIEF FOR THE DEFENDANT

Christopher M. Johnson
Chief Appellate Defender
NH Bar # 15149
Appellate Defender Program
10 Ferry Street, Suite 202
Concord, NH 03301
603-224-1236

(15 minutes oral argument)

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QUESTIONS PRESENTED

1. As a matter of statutory interpretation, does the consent of an adult patient defeat a charge brought under RSA 632-A:2, I(g)(1)?
2. Is RSA 632-A:2, I(g)(1) unconstitutionally vague, for including an element that the defendant act "in a manner or for purposes which are not professionally recognized as ethical or acceptable?"
3. Is consent a defense to a charge brought under RSA 632-A:2, I(i)?

All issues raised in interlocutory statement.

TEXT OF RELEVANT AUTHORITIES

RSA 632-A:2, I(g) and I(i) - AFSA

> I. A person is guilty of the felony of aggravated felonious sexual assault if such person engages in sexual penetration with another person under any of the following circumstances:
>
> . . .
>
> (g) When the actor provides therapy, medical treatment or examination of the victim and in the course of that therapeutic or treating relationship or within one year of termination of that therapeutic or treating relationship:
>
> (1) Acts in a manner or for purposes which are not professionally recognized as ethical or acceptable; or
>
> (2) Uses this position as such provider to coerce the victim to submit.
>
> . . .

(i) When the actor through concealment or by the element of surprise is able to cause sexual penetration with the victim before the victim has an adequate chance to flee or resist.

RSA 626:6 – Consent

I. The consent of the victim to conduct constituting an offense is a defense if such consent negatives an element of the offense or precludes the harm sought to be prevented by the law defining the offense.

II. When conduct constitutes an offense because it causes or threatens bodily harm, consent to the conduct is a defense if the bodily harm is not serious; or the harm is a reasonably foreseeable hazard of lawful activity.

III. Consent is no defense if it is given by a person legally incompetent to authorize the conduct or by one who, by reason of immaturity, insanity, intoxication or use of drugs is unable and known by the actor to be unable to exercise a reasonable judgement as to the harm involved.

STATEMENT OF THE CASE AND FACTS

The State charged Robert Allore with two counts of aggravated felonious sexual assault (AFSA) and four counts of misdemeanor sexual

assault (MSA), committed against N.C. A4-9.[1] At the time of the alleged offenses, Allore was sixty-one and worked as a visiting nurse. A284. His employer, the Rockingham Visiting Nurses Association (RVNA), assigned him to provide care to N.C., a woman then aged eighty-seven who was recuperating at home following a hospitalization for cardiac concerns. A284; H4 31 Allore visited N.C.'s home on six occasions between August 16 and September 8, 2022. A284.

The Superior Court held multiple hearings about the issues in this appeal. Both judges (St. Hilaire and Kennedy, JJ.) assigned in turn to preside issued substantive rulings about the legal significance of N.C.'s consent to sexual activity with Allore. On February 20, 2024, Judge St. Hilaire adopted in part the State's view that N.C.'s consent was not a defense and that evidence of consent would be inadmissible. Supp. 57-67. On May 8, Judge Kennedy reversed that ruling, concluding on different grounds that her consent would defeat the charges. Supp. 68-71. Later, Judge Kennedy retreated to a position of uncertainty on the question. Supp. 72-74. The legal significance of N.C.'s consent thus remains unresolved.

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[1] Citations to the record are as follows: "A" refers to the appendix containing pleadings and other relevant documents; "Supp." refers to the attached supplement containing certain orders; "H1" refers to the transcript of a suppression hearing held on Feb. 12, 2024; "H2" refers to the transcript of the final pre-trial hearing held on Feb. 22, 2024; "H3" refers to the transcript of the status conference held on March 7, 2024; "H4" refers to the transcript of the evidentiary hearing held on April 1, 2024; "H5" refers to the transcript of the final pre-trial hearing held on April 29, 2024; "H6" refers to the transcript of the motions hearing held on May 6, 2024; and "H7" refers to the transcript of the motions hearing held on May 20, 2024.

Given the interlocutory nature of the appeal, many important facts remain to be determined. For example, the State does not concede that N.C. consented. The defense does not concede that penetration occurred or that consensual sex between adults in a nurse-patient relationship would be professionally recognized as not ethical or acceptable. The trial court has made no definitive finding on these or other important disputes.

On May 20, the court convened the last hearing to date on the matter. H7 1-29. The State asked the court to adopt the position described in Judge St. Hilaire's February 20 order. H7 3, 20-22. That request, if granted, would have required the court to postpone the then-scheduled trial because jury selection was conducted under the May 8 order. H7 3-4.

The State declined a trial deposition, saying that N.C.'s present good health gave reason for confidence in her future availability despite her advanced age. The prosecutor expressed a disinclination to make her testify twice – once at a deposition and later at trial. H7 5, 9-10. The State accordingly asked the court to set a new trial date. While making that request, the prosecutor acknowledged the value of an interlocutory appeal to resolve the legal questions. H7 6-10, 18-19, 26.

The defense urged the court to adhere to the May 8 order. H7 11-15. In the alternative, the defense asked the court for an interlocutory appeal. H7 15-17.

Though it had earlier denied such a request, A84-A89, given later developments, the court endorsed an interlocutory appeal from a ruling. Supp. 74. The court made no further substantive order, choosing to defer "ruling on the issues discussed at the hearing surrounding statutory

interpretation, the consent defense, and jury instructions pending the outcome of the interlocutory appeal." Id. In due course, on June 12, 2024, the court signed an interlocutory-appeal statement. A281-A290.

This brief further describes facts and procedural history in the argument sections below. The first section addresses the proper interpretation of RSA 632-A:2, I(g)(1). The second develops Allore's constitutional challenges to RSA 632-A:2, I(g)(1). The third argues that consent is a defense to a charge under RSA 632-A:2, I(i).

SUMMARY OF THE ARGUMENT

1. As a matter of statutory interpretation, the consent of an adult patient operates as a defense to a charge under RSA 632-A:2, I(g)(1). This is true because, first, under RSA 626, even if the State can prove all the elements of the charge, the patient's consent "precludes the harm sought to be prevented by the law defining the offense." Second, and alternatively, one element of the crime – proof that the defendant acted "in a manner or for purposes which are not professionally recognized as ethical or acceptable" – can be defeated by evidence of the patient's consent.
2. The above-quoted element of RSA 632-A:2, I(g)(1) renders the statute unconstitutionally vague, especially if construed so that consent is not a defense. The phrases "professionally recognized" and "ethical or acceptable" are insufficiently definite to provide the notice due to defendants of the prohibited conduct. In particular, the phrases do not give adequate notice that the statute prohibits even consensual sex.

3. Consent also operates as a bar to conviction on charges under RSA 632-A:2, I(i). A final determination of the possible defense theories must await the introduction of evidence at trial, but Allore should not be barred, pre-trial, from eliciting evidence of N.C.'s consent to the sexual act charged under that statute.

ARGUMENT

I. AS A MATTER OF STATUTORY INTERPRETATION, EVIDENCE OF CONSENT IS ADMISSIBLE IN A CASE CHARGING A CRIME UNDER RSA 632-A:2, I(G)(1).

The AFSA indictments allege that Allore performed cunnilingus during a treatment visit on September 8, 2022. One indictment alleges that, in so doing, Allore "acted in a manner or for purposes which are not professionally recognized as ethical or acceptable." A4. The second replaces that element with the allegation that Allore used his position as a treatment-provider to coerce N.C. to submit. A5. Both charges arise under RSA 632-A:2, I(g).

In pertinent part, that statute defines as AFSA the act of engaging in sexual penetration with another

> When the actor provides therapy, medical treatment or examination of the victim and in the course of that therapeutic or treating relationship or within one year of termination of that therapeutic or treating relationship:
> (1) Acts in a manner or for purposes which are not professionally recognized as ethical or acceptable; or

(2) Uses this position as such provider to coerce the victim to submit.

RSA 632-A:2, I(g). The State charged the act of cunnilingus in the alternative under I(g)(1) and I(g)(2).

Two of the MSA charges allege that Allore kissed N.C. on the lips. The first does not allege a treating relationship. It simply states that Allore engaged in sexual contact through concealment or surprise when he kissed N.C. on the lips.[2] The second charge replaces the allegation of concealment or surprise with allegations of a medical-treatment relationship and an act not professionally recognized as ethical or acceptable. A7.[3]

The other two MSA charges prosecute alternative theories, in that each alleges that Allore had N.C. touch his penis with her hand on September 8. The charges allege misdemeanors rather than felonies because they describe sexual contact rather than penetration. RSA 632-A:4, I(a), describes misdemeanor sexual assault as occurring when the actor subjects a person over the age of thirteen to sexual contact "under any of the circumstances named in RSA 632A:2." RSA 632-A:4, I(a).

These two charges differ from each other in the same way that the two AFSA charges differ, in that each arises under one of the two sub-paragraphs of RSA 632-A:2, I(g). One alleges that Allore acted in a manner

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[2] The availability of consent as a defense to this charge is addressed in Argument III.

[3] Originally, both complaints alleged the same date – September 8, 2022, and thus charged alternative theories. The State later enlarged the time allegation of one charge to cover the period between August 16 and September 8, 2022.

or for purposes that are not professionally recognized as ethical or acceptable. A8. The other alleges that Allore used his position to coerce N.C. to submit. A9.

In June 2023, Allore gave notice of intent to assert a consent defense. A10-A12. The notice alleged facts consistent with consent and cited RSA 626:6. That statute provides that the "consent of the victim to conduct constituting an offense is a defense if such conduct negatives an element of the offense or precludes the harm sought to be prevented by the law defining the offense." RSA 626:6, I.

In December 2023, the State moved to preclude consent as a defense to charges alleging violations of RSA 632-A:2, I(g). A13-A20. In essence, the State contended that consent to sexual activity given by recipients of medical treatment is legally irrelevant when the accused was a provider of that treatment. The consent of such patients is regarded, with respect to the health-care provider, much in the way that the law treats the consent of children younger than sixteen.

Allore responded, arguing that, unlike the law's provisions for children, this statute did not broadly criminalize consensual sexual relationships between treatment providers and patients. A32-A51. Rather, it allowed adults in a treatment relationship to engage in consensual sexual activity.

Allore contended that I(g)(1) defines a crime committed in a medical or therapeutic setting when sexual acts to which the patient consents are not ethically or professionally acceptable as treatment. By way of explanation, counsel cited the notorious case of Dr. Larry Nassar. He penetrated the

Olympic gymnasts under his care, claiming to them, wrongly, that the penetration constituted appropriate medical care.[4] Their consent to the penetration, in that circumstance, does not function as a defense because they had not consented to sex, but rather only to treatment. If the penetration was not legitimate as treatment, their consent would not exculpate Nassar.[5]

Paragraph I(g)(2) defines a crime committed in a medical or therapeutic relationship when the provider uses his position to coerce the victim to consent. Allore argued that this sub-paragraph applies when the patient understands the sexual nature of the contact, as the provider does not veil its nature in a claim of treatment. The statute exists to prohibit providers from exploiting their position to obtain consent that their patients would not give, absent the exploitation. Allore argued that consent is a defense to such a charge, if that consent was freely given and not the product of manipulative coercion.

At a hearing on February 12, the court and the parties discussed the dispute. H1 76-96. On February 20, the court (St. Hilaire, J.) issued an order. Supp. 57-67. With respect to the charges under RSA 632-A:2, I(g)(1), the court adopted the State's view, concluding that the legislature

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[4] Rabin, Roni Caryn, *Pelvic Massage Can Be Legitimate, but Not in Larry Nassar's Hands* – The New York Times (Jan. 31, 2018) (last visited 12/16/24).

[5] There is a long history of sexualized "treatment" of women. See Horwitz, Rainey, "Medical Vibrators for Treatment of Female Hysteria." (Embryo Project Encyclopedia (2/29/2020) https://embryo.asu.edu/pages/medical-vibrators-treatment-female-hysteria (last visited 1/14/25). The legislature accordingly had ample reason to regulate such conduct.

intended a broad prohibition against sexual activity in the treatment context, such that consent, no matter how freely given, is as irrelevant as if the patient were a child. In so ruling, it rejected the narrower interpretation that would allow sexual activity between consenting adults, while criminalizing only the false-treatment circumstance characteristic of the Nassar case.

Next, the court turned to the charges under I(g)(2), the sub-section criminalizing the act of using one's position as a treatment-provider to "coerce the victim to submit." Supp. 65. The court concluded that "the consent of the victim would inherently negative the element of coercion because consenting to the alleged act necessarily means the victim was not coerced into submission." Supp. 66. In that sense, the court concluded that "consent is a defense to crimes charged under RSA 632-A:2, I(g)(2)." Id.

The State filed a nolle prosequi on the charges as to which the court allowed a consent defense, while going forward on the charges under RSA 632-A:2, I(i) and I(g)(1), as to which the court rejected consent as a defense. A52; H3 43. The defense moved for reconsideration. A55-A64. The court denied the motion. A64; H3 43, 59-60.

The issue resurfaced in litigation about the admissibility of expert testimony the State planned to elicit from a nurse describing the behavior considered unethical and professionally unacceptable. H3 19-25. The defense challenged the admissibility of that testimony. Id.; A65-A76. The court, in the person of a judge (Kennedy, J.) new to the case, convened a hearing on the admissibility of the opinions of the State's expert, Jessica Reeves. H4 10-97.

Reeves testified that it would be unethical for a nurse to have a sexual relationship with a patient. H4 22. When asked why, she first replied, "because it's common sense…. We don't need to be told that; it goes without saying." H4 22. She explained that Allore had no medical reason to engage in sexual activity with N.C. H4 32, 35-41.

When asked to cite the ethical rule on which she relied, Reeves first offered, "off the top of [her] head, … the Nurse Practice Act [NPA]." H4 41-43. Reeves then cited the ethics standards promulgated by the American Nurses Association (ANA). H4 43-44. She acknowledged that nurses are not obliged to join the ANA. H4 45, 79. She described some broad principles embodied in the ANA's standards, H4 45-47, such as "the preservation of the wholeness of character" so that "the patient is able to maintain autonomy and is not unduly influenced with any of the … medical decisions that they may need to make about their care." H4 45-46. She opined that the sex in this case violated those principles. H4 46-47.

On cross-examination, Reeves agreed that the NPA was "not an ethical code for nurses." H4 63-66. The idea of a one-year waiting period, for example, comes from I(g); it is not specified in the NPA. H4 67. She further testified that she based her conclusion – that Allore acted in a manner not recognized as ethical or acceptable – on the ANA code. H4 87. The court denied the defense motion to exclude Reeves's testimony. A77-A83.

On April 25, prompted by the State's recent disclosure of documents obtained from RVNA, the defense re-opened the discussion by filing a motion in limine seeking leave to introduce evidence of consent. A90-A95.

Recalling Reeves's anticipated testimony, Allore sought to introduce evidence that RVNA trained him only to avoid the sexual abuse of patients. Wholly apart from the status of consent as a formal defense, Allore argued that the statute required the State to prove both that Allore's acts were professionally and ethically unacceptable, and that Allore knew that his acts were ethically and professionally unacceptable. Because Allore's training arguably defined the boundaries of acceptable sexual activity with patients in terms of consent, evidence of that training – and thus of consent – should be admitted.

On April 29, the court convened a final pre-trial conference. H5 2-17. Counsel repeated that RVNA documents revived the relevance of consent insofar as they showed that the nursing profession's ethical standards arguably defined the boundary between ethical from unethical sexual behavior by reference to consent. H5 5. The prosecutor noted that if the court changed course on consent, the State might revive the charges previously nolle prossed. H5 8.

Following the hearing, the court issued a further order noting that the prior ruling barring consent "was premised on the assumption that the evidence at trial would show that treatment providers having sexual relationships with patients is not professionally recognized as ethical or acceptable." A97.

However, "evidence of the RVNA code of conduct casts doubt on these assumptions." A98.

The court also acknowledged the dispute about whether the State had to prove a culpable mens rea as to what the profession defined as

unethical in the realm of consensual sexual activity with adult patients. Id. In other words, did the State have to prove that Allore knew that sexual activity, even if consensual, was professionally recognized as unethical and unacceptable? Also, which professional standards controlled – those of the RVNA or those of the ANA, an organization to which Allore had not belonged? A98-A99.

By this point, the defense had advanced two arguments for the relevance of consent. First, by filing a notice of defense, Allore argued that consent, when raised, became a defense the State had to disprove. Second, by filing the later motion in limine focused on the professionally-recognized element, Allore contended that consent was intrinsically involved in an element the State had to prove to convict on the I(g)(1) charges.

On May 6, the court convened a hearing at which it orally granted Allore's motion in limine. H6 25-27. By a written order on May 8, the court ruled RVNA policies admissible and consent available as a defense theory negating an element of the crime. H6 25-26; Supp. 68-71. The order cited the dispute about the content of the nursing profession's ethical standards and that dispute's link with consent. Supp. 68-70. In essence, the court recognized that a factual dispute existed as to whether consensual sexual relationships between adults who have a provider-patient relationship are professionally recognized as ethical or acceptable. Supp. 68-71. Moreover, the court agreed with Allore that the State had to prove not only that his sexual contact with N.C. was not ethical or acceptable, but also that Allore knew that. Supp. 70. Finally, the court noted that if the State re-filed the

nolle prossed charges, the court's prior order allowing consent as a defense as to those charges would remain in effect. Supp. 71. The State then re-filed the nolle prossed charges. H6 27-28.

Soon, the tide began to turn again on the issue of consent. Supp. 72-73. The parties had litigated the admissibility of Allore's decision to surrender his nursing license. In that litigation, the parties cited the NPA, codified in RSA Chapter 326-B. Some language in the NPA suggested that even consensual sexual relations between a nurse and a patient may be unprofessional or unethical. That caused the court to wonder about the parties' prior focus on ANA standards and RVNA documents. Supp. 73. The court accordingly directed the parties to submit supplemental memos. On May 16, the defense responded. A194-A199. On May 17, the State filed a motion to reconsider the orders changing course on consent. A200-A270. On May 20, the State also responded to the May 15 order. A271-A274. Ultimately, the court declined to address the substantive issues further, instead endorsing the issues for interlocutory appeal.

The appeal presents first questions of statutory interpretation. This Court is "the final arbiter of legislative intent as expressed in the words of the statute considered as a whole." State v. Czekalski, 169 N.H. 732, 737 (2017). The Court will first examine the language found in the statute and ascribe the plain and ordinary meanings to the words used. If the statute's language is clear and unambiguous, [the Court] does not look beyond it to discern legislative intent. Furthermore, [the Court] interpret[s] statutes in the context of the overall statutory scheme and not in isolation. [The Court's] goal is to apply statutes in light of the legislature's intent in

enacting them, and in light of the policy sought to be advanced by the entire statutory scheme. Id. (cleaned up). On appeal, this Court decides such questions de novo. Attorney General v. Hood, 2025 N.H. 3, ¶9.

When a statute is ambiguous or silent on a point, this Court will consider other sources to aid in interpretation. In particular, "[i]f a statute is ambiguous, [the Court] consider[s] legislative history to aid [its] analysis." State v Whittey, 149 N.H. 463, 467 (2003).

Finally, if those tools do not resolve the question, this Court will apply the rule of lenity. That rule "serves as a guide for interpreting criminal statutes where the legislature failed to articulate its intent unambiguously." State v. Dansereau, 157 N.H. 596, 602 (2008) (cleaned up). The rule "generally holds that ambiguity in a criminal statute should be resolved against an interpretation which would increase the penalties or punishments imposed on a defendant." Id. It has its source in "the instinctive distaste against men languishing in prison unless the lawmaker has clearly said they should." Id. In applying the rule, this Court rejects "the impulse to speculate regarding a dubious legislative intent and avoid[s] playing the part of a mind reader." Id.

As noted, the statutory-interpretation dispute about consent traveled two separate paths in the trial court. First, taking RSA 626:6 as a starting point, the defense argued that N.C.'s consent serves as a defense, even if the State could otherwise prove the elements of the charges. Second, taking RSA 632-A:2, I(g)(1)'s elemental language – not professionally recognized as ethical or acceptable – as a starting point, the defense argued that, to prove that element, the State had to disprove N.C.'s consent.

In a certain sense, the distinction between the paths does not matter. If the defense view on either were correct, the trial court would arrive at essentially the same destination. That is, the success of either argument would mean that evidence bearing on consent would be relevant and therefore admissible at trial. Also, once the issue of consent is raised, the State bears the burden of disproving consent or of proving that professional recognition of the acceptability of sex between a nurse and patient did not depend on consent. In that sense, the characterization of the issue as a defense the State must disprove, or as intrinsic in an element the State must prove, makes little functional difference. State v. Cooper, 135 N.H. 258, 261 (1992).

However, the distinction between the two paths to that common destination remains analytically useful because each implicates different arguments. Indeed, the trial court trod both paths at different times in this case. When considering the matter under RSA 626:6, Judge St. Hilaire concluded that N.C.'s consent was not relevant. When considering the matter by reference to the statutory "professionally-recognized" element, Judge Kennedy found consent relevant, depending on the governing professional standard. Section A below presents the argument that consent is a defense to a charge brought under RSA 632-A:2, I(g)(1). Section B, in the alternative, presents the argument that disproof of consent is intrinsic to an element of the crime.

A. Consent is a defense to a charge under I(g)(1).

The statute defines as AFSA the act of engaging in sexual penetration with another person

> When the actor provides therapy, medical treatment or examination of the victim and in the course of that therapeutic or treating relationship or within one year of termination of that therapeutic or treating relationship:
> (1) Acts in a manner or for purposes which are not professionally recognized as ethical or acceptable; or
> (2) Uses this position as such provider to coerce the victim to submit.
> RSA 632-A:2, I(g).

In raising a consent defense, Allore cited, among other points, RSA 626:6, captioned Consent. That statute provides:

> I. The consent of the victim to conduct constituting an offense is a defense if such consent negatives an element of the offense or precludes the harm sought to be prevented by the law defining the offense.
> II. When conduct constitutes an offense because it causes or threatens bodily harm, consent to the conduct is a defense if the bodily harm is not serious; or the harm is a reasonably foreseeable hazard of lawful activity.
> III. Consent is no defense if it is given by a person legally incompetent to authorize the conduct or by one who, by

> reason of immaturity, insanity, intoxication or use of drugs is unable and known by the actor to be unable to exercise a reasonable judgment as to the harm involved.
> RSA 626:6; see also State v. Foss, 148 N.H. 209 (2002) (construing statute in AFSA coercion context).

Allore argued that consent would preclude the harm sought to be prevented by the statute, postulating that RSA 632-A:2, I(g)(1) did not intend to prevent sexual activity between consenting adults. The State argued that the legislature intended to do exactly that. Several considerations support Allore's interpretation.

First, in several statutes, the legislature has made clear its intent to eliminate consent as a defense. For example, RSA 632-A:2, I(k), defines two variants of AFSA, committed in a school setting or when an actor holds a position of authority over the victim. The statute specifies that "[c]onsent of the victim under any of the circumstances set forth in subparagraph (k) shall not be considered a defense." RSA 632-A:2, I(k)(3). A similar provision appears in the paragraph covering sexual activity between corrections officers or probation and parole officers (PPO) on the one hand, and inmates or persons under PPO supervision, on the other. See RSA 632-A:2, I(n) ("Consent of the victim under any of the circumstances set forth in subparagraph (n) shall not be considered a defense").

A well-established canon of statutory interpretation teaches that courts should not add language to statutes that the legislature did not see fit to enact. State v. Dor, 165 N.H. 198, 201 (2013). This holds especially true when the legislature enacted that language in other parts of the statute. State

v. Mayo, 167 N.H. 443, 452 (2015). Had the legislature intended to bar the consent defense in paragraph (g)(1), it would have said so.

Second, if the State were correct that the patient's consent were legally irrelevant under I(g)(1), there would be no reason to enact I(g)(2). That paragraph criminalizes the act of one who, while serving as a treatment-provider, "[u]ses this position as such provider to coerce the victim to submit." RSA 632-A:2, I(g)(2). Paragraph I(g)(2) covers the circumstance in which a treatment-provider engages in non-consensual sexual activity with a patient. Indeed, in finding consent to be a defense to a charge under I(g)(2), Judge St. Hilaire so ruled. However, if consent is no defense under I(g)(1), the legislature would have no need to enact I(g)(2). Sexual activity, whether consensual or not, between a provider and a patient would already be criminal under I(g)(1), leaving the legislature no need further to specify that coerced submission to sex in the provider-patient relationship is criminal. This Court avoids interpretations of statutes that render a part of the statute superfluous. State v. Duran, 158 N.H. 146, 155 (2008).

Third, the plain language of I(g)(1) supports Allore's interpretation. Allore contended that the statute aimed to criminalize acts of penetration or sexual contact that purport to be treatment, unless the treatment is professionally recognized as ethical or acceptable. Examples illustrate the scope of the statute under that interpretation. An OB/GYN might engage in an act constituting digital penetration. RSA 632-A:1, V, defines "sexual penetration" as "any intrusion, however slight, of any part of the actor's body … into genital or anal openings of the victim's body." The statute

does not require proof of a purpose of sexual gratification. Cf. RSA 632-A:1, IV (defining "sexual contact" in part in terms of a purpose of sexual gratification). A prosecution of an OB/GYN under I(g)(1) would turn on whether the act was "professionally recognized as ethical or acceptable." If it was not, the provider committed AFSA. If it was, the provider committed no crime.

With the crime thus narrowly defined, a patient's consent, in this circumstance, does not exculpate the defendant. If the patient consented to an act she believed to be professionally appropriate treatment, the provider's guilt depends on whether the penetrative act was professionally recognized as ethical or acceptable. The Nassar example illustrates that point.

A second example adds further clarity. A person acting as a sex surrogate[6] in sex therapy might engage in sexual penetration with the patient. Assuming the patient factually consented to that sexual penetration, a prosecution of the surrogate under I(g)(1) would depend on whether the surrogate acted in a manner professionally recognized as ethical or acceptable.

The State first cited State v. Hollenbeck, 164 N.H. 154 (2012). That case involved the prosecution of a psychologist under I(g)(1) and reached this Court pre-trial, on a State's appeal of a ruling dismissing the charges on constitutional grounds. Hollenbeck, 164 N.H. at 156. The State alleged that

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[6] See generally Wikipedia, *Surrogate Partner*, https://en.wikipedia.org/wiki/Surrogate_partner (last visited January 17, 2025).

Hollenbeck became sexually involved with a former patient less than a year after the therapeutic relationship ended.

In the trial court, Hollenbeck raised a constitutional challenge to the statute, arguing that the legislature lacked constitutional power to criminalize consensual sexual conduct between adults. Among other authorities, he relied on Lawrence v. Texas, 539 U.S. 558 (2003), which invalidated a Texas statute prohibiting consensual homosexual sex. The New Hampshire trial court agreed and dismissed the case.

Given the pre-trial posture, it was assumed both that the victim consented to sex with Hollenbeck and that the statute prohibited the sexual acts regardless of consent. Thus, neither Hollenbeck nor the State made arguments about the proper interpretation of the statute. Both the majority and dissenting opinions in Hollenbeck accordingly shared that assumption. Hollenbeck, 164 N.H. at 159 (majority); at 165-66 (dissent). Because the disputed constitutional question depended on that assumption, neither the majority nor the dissent had occasion to apply canons of statutory interpretation to test it.

In that context, the Hollenbeck majority stated that "lack of consent is irrelevant; it is not an element of the crime for which the defendant was charged." Id. at 159. That statement, reflecting an assumption embedded in the litigation and not disputed by the parties, does not constitute a holding binding this Court now.

The principal statutory feature advanced against Allore's interpretation, and relied on by Judge St. Hilaire, is the one-year waiting period. The State argued that if the statute just covered acts of penetration

occurring in the guise of treatment, I(g)(1) would contain no provision covering acts committed "within one year of termination of that therapeutic or treating relationship."

At the outset, it bears emphasis that (g)(1) does not contain that language. Rather, it appears in the general part of I(g) preceding sub-division into (g)(1) and (g)(2). The within-one-year clause was enacted at the same time the legislature added the prohibition codified in (g)(2), some years after it established the rule now codified in (g)(1). The placement of the "within-one-year" language in I(g)'s general part rather than in (g)(2), thus, reflects only the statute's grammatical structure, in that the general part specifies the "treatment relationship" that the "one-year" provision modifies. The (g)(1) and (g)(2) sub-paragraphs, in turn, describe the prohibited acts.

Legislative history supports the conclusion that the "one-year" extension clause applies only to (g)(2). In 1975, the legislature replaced the statute defining the crime of "rape" with RSA Chapter 632-A, defining crimes of "sexual assault." Laws 1975, ch. 302:1. RSA 632-A:2, VII defined as AFSA the act of sexual penetration "[w]hen the actor engages in the medical treatment or examination of the victim in a manner or for purposes which are not medically recognized as ethical or acceptable." No analog to I(g)(2) nor any waiting period existed in that statute. By its plain terms, that statute covered an act committed during the "medical treatment or examination of the victim." The lawfulness of that act depended on whether the treatment or examination was "medically recognized as ethical or acceptable."

The only prosecution reviewed in a published opinion under that early version of the statute does not contradict that interpretation. In State v. vonKlock, 121 N.H. 697 (1981) (overruled on other grounds, State v. Smith, 127 N.H. 433, 438 (1985)), the State charged a psychologist "with committing [AFSA] while counseling one of his patients." Id. at 698. VonKlock argued, among other points, that the State presented insufficient evidence "that the defendant was engaged in medically treating the victim at the time of the assault." Id. at 699. Specifically, he claimed that the statute did not reach his conduct because he was engaged in psychology, not medicine. This Court rejected that argument. Id. at 700. More to the point here, the evidence at trial alleged an offense committed while vonKlock counseled the patient. Id. at 699.

In 1992, the legislature re-structured the paragraph to add "therapy" to it. N.H. Laws 1992, ch. 254:6. With new language in italics and repealed language in strikethrough, the statute prohibited sexual penetration

> (g) When the actor engages in the provides therapy, medical treatment or examination of the victim in a manner or for purposes which are not medically professionally recognized as ethical or acceptable.
>
> RSA 632-A:2, I(g) (Supp. 1993).

There is no indication that the legislature intended to do anything other than expressly foreclose vonKlock's argument excluding mental-health professionals from the statute's scope.

In 1998, the legislature revised the statute again. N.H. Laws 1998, ch. 240:2. It added I(g)(2) and put the original prohibition in a new I(g)(1). Also, the legislature added the one-year provision. With new language in italics, the statute prohibited sexual penetration

> (g) When the actor provides therapy, medical treatment or examination of the victim and in the course of that therapeutic or treating relationship or within one year of termination of that therapeutic or treating relationship:
> (1) Acts in a manner or for purposes which are not professionally recognized as ethical or acceptable; or
> (2) Uses this position as such provider to coerce the victim to submit.
> RSA 632-A:2, I(g).

Committee reports confirm that the focus of concern was sex between mental-health therapists and patients. Leg. His. at 28-35.[7] An initial proposal would have removed the reference to "therapy" added in 1992, thereby returning I(g) to something close to its original form while creating a new subsection specific to mental-health counselors. Id. at 34, 229. That new subsection would have defined, as AFSA, penetration "[w]hen the actor is a mental health counselor and the victim is a mental health client of the mental health counselor. Consent on behalf of the

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[7] Legislative history is available on the General Court's webpage: HB730H.pdf (last visited January 11, 2025). The document contains 242 pages. Citations to "Leg. His." are to page numbers in that document. The appendix contains a condensed version. A291-A312.

mental health client is not a legal defense." A295. The bill defined "mental health client" as a person "who is receiving or has received mental health counseling in the past year." A295. The proposal would thus have prohibited even consensual sex in that therapist-patient setting.

During the legislative process, the idea of a mental-health-specific statute was abandoned in favor of integrating the new prohibition into I(g), now sub-divided into a (g)(1) and a (g)(2). A299. Committee records indicate that the change was motivated by a concern about the difficulty of adequately defining "mental health counselors." A310-A311. With that change to a generally applicable statute rather than a mental-health-specific statute, the legislature defined the new (g)(2) in terms of "coercion" and dropped the proposal prohibiting consent defenses.

In several respects, this history supports Allore's interpretation. First, the legislature considered and rejected a proposal explicitly prohibiting even consensual sex between therapists and patients. It ultimately decided to define the prohibition in terms of coercion. See United States v. Howe, 167 N.H. 143, 151 (2014) ("legislative history can consider part of a statute that never came into existence because, for instance, the language under question was rejected by the legislature").

Second, there is no sign that the legislature intended to change the

statute's long-standing focus on acts committed in the guise of treatment.[8] This Court has held that, "unless the context indicates otherwise, words or phrases in a provision that were used in a prior act pertaining to the same subject matter will be considered in the same sense." In re Williams, 159 N.H. 318, 324 (2009). The legislature has not changed the language of (g)(1) in any way indicating an intent to cover acts not justified by the provider as "treatment."

Finally, to the extent that the various components of the statute leave the question ambiguous and unresolved by the text and legislative history, this Court must rely on the rule of lenity. That rule requires that consent be allowed as a defense.

B. Consent is intrinsic to an element of I(g)(1).

If the Court does not adopt the reasoning in Section A, it must then consider whether an element of RSA 632-A:2, I(g)(1), makes N.C.'s consent relevant. The argument focuses on the requirement that the State prove that Allore's acts were "not professionally recognized as ethical or acceptable."

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[8] At an April 1997 Senate committee hearing on a similar proposal, County Attorney Soldati expressed his understanding that the 1992 statute defined as AFSA "to engage in sexual penetration with a person if they are engaged in therapy with you and that is used as 'you know, this is part of your therapy" type of thing." HB735S.pdf (at 8); see also A41. He added that "the current statute requires some showing that they used their position as a therapist or a medical treatment and engaged in sexual penetration as if that was part of that treatment. And the only extension that the current bill provides is the use of that authority position … to coerce." Id. at 9.

In orders in early May, the court ruled that RVNA policies would be admissible and that consent could negate an element of the charged crime. H6 25-26. The court acknowledged the factual dispute about the nursing profession's ethical standards and that dispute's link with the relevance of consent. Supp. 68-70. The court later reconsidered, to the extent of authorizing an interlocutory appeal. Several considerations support the conclusion that the "professionally recognized" element makes N.C.'s consent relevant.

First, I(g)(1) codifies an element the State must prove. Due Process bars courts from directing a verdict against a defendant in a criminal case. State v. Williams, 133 N.H. 631, 634-35 (1990). That principle holds not only for the elements collectively that define a crime, but also for each individual element. Thus, a trial court errs if, in the absence of a defense stipulation, it instructs the jury to find proven any element of a charged crime.

Second, the element delegates to professional recognition the task of determining the governing standard. Professional recognition will distinguish between behavior that is "ethical or acceptable" and behavior that is not. The statute, though, defines none of those terms. Having enacted no clarifying definitions, the legislature evidently intended the finder of fact to ascertain the governing professional standards.

Third, given that ascertaining the governing professional standards calls for a finding of fact, the State can prevail in its effort to exclude the issue of consent from the case only if no reasonable factfinder could conclude that consent was relevant under the applicable professional norms.

The standard of relevance sets a low bar. Evidence is relevant if it has "any tendency to make a fact more or less probable than it would be without the evidence." N.H. R. Ev. 401.

Here, Reeves, the State's expert, after first citing "common sense," relied principally on the ANA standards to conclude that even consensual sex between nurses and patients is prohibited. The ANA, though, is a voluntary organization and its authority to define professional standards is subject to factual dispute.

Later, the State emphasized the NPA. See RSA Chapter 326-B. That chapter established a Board of Nursing and defined its powers. It further defined the scope of practice and authority of various categories of nurses. RSA 326-B:11 to :14-a. Reeves acknowledged that the NPA is not an ethics code for nurses.

The court focused on two NPA provisions. Supp. 72.

RSA 326-B:34, I establishes a licensed nurse's duty to tell a patient (or "client," to use the NPA's term) of the licensee's duty to report to the Board information indicating that a licensed nurse "has engaged with the client in sexual relations." The provision adds a duty to "inform the client that the act reported by the client may be unprofessional or unethical and may subject the actor to disciplinary action by" the licensing authority. RSA 326-B:34, I (emphases added).

RSA 326-B:37 codifies circumstances in which the Board can discipline a licensee. Paragraph II(m) contains a diverse list of grounds, including "failure to maintain professional boundaries with clients…," "… abuse of any client," and "engaging in sexual conduct with a client,

touching a client in a sexual manner, requesting or offering sexual favors or language or behaviors suggestive of same. . . ." RSA 326-B:37, II(m). Here again, the statute declares that the Board "may discipline a licensee" for the specified acts.

The statute contains no express exclusion for consensual sexual relations. The defense emphasized that the legislature's use of the word "may" suggests that sexual activity between a nurse and patient, depending on the circumstances, might not be unethical or unprofessional. The boundary may depend on consent.

The defense called the court's attention to RVNA documents. A92-A94. In its May 8 order, the court cited records suggesting "that only sexual 'abuse' of patients is prohibited, as opposed to all sexual relationships." Supp. 68. The court noted that, otherwise, the RVNA documents "are silent on the issue of consensual sexual relationships with patients." Supp. 69.

Allore does not claim that, as a matter of law, consent defines the statutory boundary between professional and unprofessional sexual activity in this context. Rather, he agrees with the May 8 ruling that it was not the court's place to decide the issue, for it "will ultimately be up to the jury to resolve." Supp. 70. It is likewise not this Court's place to decide a factual issue.

Allore is entitled, at trial, to elicit evidence that consent, in whole or in part, defines the boundary between acceptable and unacceptable sexual relationships with patients. If he does so, it will become the State's obligation to prove the contrary, or to prove that N.C. did not in fact

consent. Before trial, on this factual question, this Court can say nothing more.

II. RSA 632-A:2, I(G)(1) IS UNCONSTITUTIONALLY VAGUE.

Allore argued that I(g)(1) is vague. A45-A48. The problem resides in the not-professionally-recognized-as-ethical-or-acceptable element. In its February 20 order, the court rejected Allore's constitutional arguments in a footnote, citing Hollenbeck and the policy of not deciding constitutional questions that can be avoided. Supp. 63 at n1. In the May 8 order, the court ruled in Allore's favor on statutory grounds and did not address the constitutional arguments. Supp. 68-71.

The void-for-vagueness doctrine rests on the right to due process embodied in Part I, Article 15 of the New Hampshire Constitution and the Fifth and Fourteenth Amendments of the Federal Constitution. State v. Pike, 128 N.H. 447, 451 (1986). A criminal statute "can be impermissibly vague for either of two independent reasons." State v. MacElman, 154 N.H. 304, 307 (2006). First, "a statute proscribing conduct [must] not be so vague as to fail to give a person of ordinary intelligence a reasonable opportunity to know what is prohibited." In re Justin D., 144 N.H. 450, 453 (1999) (quoting State v. Winslow, 134 N.H. 398, 399 (1991)). A statute fails to give fair notice if "men of common intelligence must necessarily guess at its meaning and differ as to its application." Connally v. General Construction Co., 269 U.S. 385, 391 (1926); State v. Beckert, 144 N.H. 315, 319 (1999). A statute must also "give ascertainable standards of guilt

to the judge and jury charged with making an objective determination of guilt." State v. Albers, 113 N.H. 132, 134 (1973) (cleaned up).

Second, by leaving law enforcement officers with unfettered discretion, vague laws pose a danger of "arbitrary and erratic convictions." State v. Glidden, 122 N.H. 41, 46 (1982). The doctrine invalidating vague laws accordingly seeks to guarantee at least "minimal guidelines to govern law enforcement." Kolender v. Lawson, 461 U.S. 352, 358 (1983) (cleaned up).

This Court presumes the constitutionality of statutes, and places upon parties challenging a statute a "heavy burden" of demonstrating its unconstitutionality. Webster v. Town of Candia, 146 N.H. 430, 434 (2001). This Court reviews questions of constitutional law de novo. MacElman, 154 N.H. at 307.

As far as counsel can tell, no other state defines a crime in terms of what is "professionally recognized as ethical or acceptable." A few statutes define fraud crimes in part by reference to "professionally recognized standards." See, e.g., UT ST §26B-3-1106(1)(e) (Person may not claim medical benefit "for items or services which the person or the provider knew were not medically necessary in accordance with professionally recognized standards"); KS ST 21-5927(a)(1)(F) (similar, using phrase "professionally recognized parameters"). New Hampshire has such a statute. See RSA 167:61-a, I(d) (barring claim for payment for a good or service "which is not medically necessary in accordance with professionally recognized standards").

Both parts of I(g)(1) contribute to its vagueness. The statute defines neither "professional recognition" nor "ethical or acceptable." Those terms invite ambiguity. Do the standards of the ANA, the RVNA, or the NPA control? Or, as Reeves put it, is the impermissibility of consensual sex in this context a matter of "common sense?" What is meant by "ethical or acceptable?" Could consensual sex be not ethical but still "acceptable?" The statute does not answer these questions.

Federal courts have considered vagueness challenges to statutes using the phrase "professionally recognized." In Association of Am. Physicians and Surgeons v. Weinberger, 395 F. Supp. 125 (N.D. Ill. 1975), aff'd per curiam 423 U.S. 975 (1975), an association of doctors challenged a federal law governing the Medicare and Medicaid programs. A provision allowed payment only for services that were "medically necessary," constituted "proper care," and were "consistent with professionally recognized health care standards." Id. at 129.

The challenge focused on the above-quoted phrases and the court acknowledged that they "are not highly specific." Id. at 138. Nevertheless, citing the principle that "the Constitution does not require impossible standards," the court rejected the challenge. The words "convey[] sufficiently definite warning as to the proscribed conduct when measured by common understandings and practices." Id. In so ruling, the court distinguished cases invalidating criminal statutes as vague, because the "present statutory scheme does not impose criminal sanctions." Id.

In Appeal of Plantier, 126 N.H. 500, 513-14 (1985), this Court considered a challenge to a statute authorizing the medical licensing board

to take disciplinary action against a doctor who "has engaged in dishonest, unprofessional or immoral conduct." While agreeing that the phrase "immoral conduct" does not "lend itself to definite boundaries by which it can be marked," this Court rejected Plantier's vagueness challenge. The Court noted that the "forms which immoral conduct may take are numerous and varied, making it virtually impossible for the legislature to set forth all of the acts which come within the meaning of the phrase." Id. at 513.

In addition, this Court noted that courts are "reluctant" to invalidate statutes as vague when the statute "applies without question" to the litigants in the case. Id. That principle defeated the claim insofar as a "person of ordinary intelligence certainly would know that it would be 'immoral' for a physician to perform fellatio on and masturbate minor male patients during … a physical examination." Id.

Those rationales cannot defeat Allore's challenge. First, this is a criminal case, requiring a higher standard of clarity than is required of a civil statute. In Village of Hoffman Estates v. Flipside, 455 U.S. 489, 498-99 (1982), the Court explained that it has "expressed greater tolerance of enactments with civil rather than criminal penalties because the consequences of imprecision are qualitatively less severe." See also Varandani v. Bowen, 824 F.2d 307, 312 (4th Cir. 1987) (noting higher standard of clarity required of criminal statutes).

Second, a statute regulating medical benefits must cover many kinds of medical procedures. A degree of generality of expression is, therefore, unavoidable. The Plantier Court made that point in the medical-licensing context. In Varandani, the court noted the need, in the medical-benefits

context, for a standard based on what "other health professionals consider to be acceptable standards of health care." Id. (analogizing statutory standard to "reasonable man" standard).

By contrast, Allore's argument does not imply that the legislature must enact an impossibly detailed statute. Rather, his claim poses one specific question that the legislature could directly answer, as it has in other AFSA paragraphs: does a provider commit a crime when he has consensual sexual relations with a patient? Some states, in no uncertain terms, criminalize consensual sex in the provider-patient setting. See, e.g., Iowa Code §709.15(2)(c) (defining sexual exploitation as "any sexual conduct with a patient or client or former patient or client within one year of the termination of the provision of mental health services… for the purpose of arousing or satisfying the sexual desires of the counselor or therapist or the patient or client….").

Instead of enacting a clear prohibition of that sort, the New Hampshire statute leaves the legal significance of consent open to doubt depending on what may be "professionally recognized as ethical or acceptable" at the relevant time. A statute that merely prohibits sexual activity that is "not professionally recognized as ethical or acceptable" dodges, rather than answers, the question about consent.

There is yet a further problem. In the medical-benefits context, statutes do not delegate to the medical profession quasi-legislative power over a question of law. Rather, such statutes' "professionally recognized" language establishes a kind of reasonable-person standard suitable for the

technical, expertise-influenced determination of what treatments are medically necessary and appropriate for a given patient.

By contrast, whether nurses and patients may have consensual sex does not pose a question best answered by professional expertise. It demands, rather, a policy and value judgment properly decided by the legislature. By committing the question to professional expertise, the statute fails to give defendants fair notice of what the law prohibits. A defendant must be presumed not only to know the statute, but also the relevant professional consensus.

Finally, unlike Plantier, this case does not arise out of facts that unquestionably fit within any plausible construction of the challenged statutory term. Reasonable people could differ about whether sex between consenting adults is always unethical or unacceptable when the adults have also a provider-patient relationship.

For these reasons, this Court must declare RSA 632-A:2, I(g)(1) unconstitutionally vague if the statute is construed to permit conviction even when the patient consents. Thus, this Court should, in construing the statute, apply the canon that statutes should, where possible, be interpreted to avoid making them unconstitutional. Guillou v. State, Div. of Motor Vehicles, 127 N.H. 579, 580-81 (1986).

III. THE COURT MUST ADMIT EVIDENCE OF CONSENT IN DEFENSE OF CHARGES ARISING UNDER I(I).

One MSA charge alleges that Allore kissed N.C. on the lips and, in so doing, engaged in sexual contact through concealment or surprise in

violation of RSA 632-A:2, I(i). A6. Allore proposed to rely on consent as a defense to this charge. The State objected, denying that consent would be a defense to the charge. A19. The defense responded. A21-A31. Citing various canons of statutory interpretation, the defense argued that the legislature recognized consent as a defense to charges under I(i).

On February 20, the court concluded that consent was not a defense. Supp. 59-62. In so ruling, the court reasoned that if the jury found the element of concealment or surprise, it necessarily would find that the victim did not consent. Finding that consent on the one hand, and concealment or surprise on the other, represented mutually inconsistent accounts of the facts, the court concluded that the defense could present "a theory of consent as an alternative set of facts at trial," but that consent does not otherwise constitute a defense. Id. The defense filed a motion to reconsider. A63-A64. The court denied that motion. A64; H3 43, 59-60.

Allore incorporates by reference the points and authorities in Argument I above. He makes the same initial argument here as in Section I(A) that the legislature has explicitly withdrawn the consent defense in some paragraphs of RSA 632-A:2, I, but not in I(i). Moreover, as with subparagraph I(g)(1), this Court must, upon the survival of ambiguity after using other tools, rely ultimately on the rule of lenity.

The analysis differs somewhat as to the application of RSA 626:6, I's principle that consent constitutes a defense "if such consent negatives an element of the offense or precludes the harm sought to be prevented by the law defining the offense." In State v. Ramos, 149 N.H. 272 (2003), this

Court decided the appeal of a conviction under I(i) raising an issue of consent.

There, the State charged that Ramos engaged in sexual penetration with a sleeping woman. Ramos claimed that they had consensual sex while she was awake. Id. at 272-73. The trial court denied Ramos's request for a consent instruction, reasoning that, "for the jury to find that penetration occurred reasoning that, "for the jury to find that penetration occurred only through the defendant's concealment or by surprise, it necessarily would have to find that the victim did not consent." Id. at 273. This Court affirmed.

It noted, first, that Ramos had offered evidence to support consent, through his own testimony. Id. at 274. That circumstance potentially distinguishes Ramos, insofar as the trial court here proposes to bar Allore from introducing evidence of N.C.'s consent.

The Ramos Court relied on the distinction between a theory of defense and a theory of the case. Id. Ramos made the case a test of the credibility of two versions of the facts – the victim's (in which he penetrated her while sleeping), and his (in which she was awake and consenting). This Court agreed with the trial court's reasoning:

> As the trial court correctly realized, if the jury found the State proved all elements of the charged variant of [AFSA], it would have to find the victim did not consent. Likewise, if the jury found the State did not prove the element of concealment or surprise, the jury would be required to acquit the defendant.

Id. at 276.

In a sense, a decision is premature, insofar as it is uncertain what evidence will be elicited at trial. There are also circumstances in which concealment or surprise could be consistent with consent. Allore contends only that he should not be barred, pre-trial, from eliciting evidence of N.C.'s consent to the sexual act charged under RSA 632-A:2, I(i). He contends moreover that if, like Ramos, he elicits evidence of consent, the State must prove, beyond a reasonable doubt, that he had sexual contact with N.C. by concealment or surprise and without consent.

CONCLUSION

WHEREFORE, Mr. Allore respectfully requests, on the authority of the first and third arguments, that this Court remand with instructions to allow him to present evidence of consent. He requests, on the authority of the second argument, that the Court dismiss the charges brought under RSA 632-A:2, I(g)(1).

Undersigned counsel requests fifteen minutes of oral argument before a full panel.

The appealed decisions were in part in writing and to that extent are appended to the brief.

This brief complies with the applicable word limitation and contains approximately 9230 words.

January 17, 2025

Respectfully submitted,

/s/ Christopher M. Johnson
NH Bar # 15149
Chief Appellate Defender
Appellate Defender Program
10 Ferry Street, Suite 202
Concord, NH 03301
(603) 224-1236
cjohnson@nhpd.org

CERTIFICATE OF SERVICE

I hereby certify that a copy of this brief is being timely provided to the Criminal Bureau of the New Hampshire Attorney General's office through the electronic filing system's electronic service.

/s/ Christopher M. Johnson